

Genesis of Health Care (HC) Debate: Problems and Choices

In most countries, the HC system is a mix between the market and the government. In recent years, this system has produced some remarkable accomplishments. Many terrible diseases such as small-pox, polio, leprosy, malaria, tuberculosis, cholera, typhoid, etc have either been eradicated or curative treatment/ drugs have been made available. Life expectancy and infant mortality rates have improved in most countries more from 1950 to 1990 than during the entire span of recorded history.

Advances in health technologies, from laparoscopy to MRI to genetic engineering and antiretroviral drugs, cheaper Beta-blockers to stroke-busters, chemotherapy for cancer, steroids for skin diseases, psychotropic drugs for mental health problems, protease inhibitors for AIDS, all have enabled people to live pain-free and more productive lives. This discovery of the germ theory of diseases a better understanding of hygiene and the development of antibiotics and vaccines and insulin were more instrumental to reduce population mortality since 1750 till to date. Some of these drugs represent alternatives to more invasive surgical procedures. Others are used in conjunction with surgical and radiation treatments. Still, others provide treatment for conditions where no treatment was available previously.

Evidence of cross-country growth regression suggests that the contribution of health to GDP is large. Indeed, the health of a population has been identified as one of the most robust and potent drivers of economic growth. One study reveals that an extra year of life expectancy raises steady-state GDP per capita by about 0.4 percent.

Even with all these great achievements, major health problems remained unsolved, and great disparities still exist between rich and poor; developed countries on the average spend more than US \$1200 per capita for HC, whereas, LDCs spend less than US \$13 per capita on HC.

Also, communicable diseases like hepatitis, HIV/STD/AIDS, recurrence of drug-resistant malaria, and tuberculosis are spreading. Many avoidable deaths are on the rise because of misdiagnosis, mistreatment, occupational hazards, and accidental deaths. The devastating economic impact of AIDS increased from 40 to 50 years between 1960 to 1990, now only due to this epidemic, LE has reversed and declined to 46 years in 2001. Moreover, many lifestyle diseases are on the rise such as diabetes, cardiovascular diseases, obesity, etc.

Problem Identified

[Ref: Victor Fuchs, "Who Shall Live?"]

Three major problems are identified that have made HC system itself sick:

1. A Crisis of Cost:

Costs of HC services as a percentage of GDP are rising all over the world. HC is about 8 percent of the world's income, making it one of the largest industries in the global economy. Moreover, the resource gap is widening between rich and poor countries,

despite commitment at the UN Summit, 2000 at the MDG declaration by the OECD countries. The major causes of cost escalation in HC spending are as follows:

- i. demographic (ageing population)
- ii. information (educated consumers)
- iii. standard of living (QoL expectations)
- iv. relative price effect (skill-intensive specialists)
- v. structures (incentives)
- vi. life-style (abuses)
- vii. innovation (technology)

Payments for HC services take many forms: fee-for-services (out-of-pocket expenses), insurance premiums on taxes.

Bangladesh NHA-2001 reports that Bangladeshi people spend about US \$12 per capita on health. Only 35 percent (\$4.2) is spent by the public sector from tax revenues and donor's funds and the remaining 65 percent (\$7.8) is spent by private OOPs. At the same time, a bigger section of people often are unable to pay for HC during the acute need for treatment. Health-seeking behavior observed in Bangladesh suggests that only 15 percent of people get access to appropriate providers, where 11.4 percent are treated by the private clinics and only 3.6 percent have access to public facilities.

Reform proposals to contain costs

- i. To change in supply to driven down price and ultimately costs. This means a substantial increase in the number of hospitals and doctors.
- ii. Reduce utilization by improving health of the population. This approach argues that more preventive medicine, health education, environmental improvements could reduce the need for hospitals, physicians, drugs and diagnostic centers.
- iii. Administrative control and planning to contain costs.
- iv. Compulsory health insurance schemes for risk-sharing with substantial deductibles and coinsurances.
- v. Employ physician on a capitation basis rather than a fee-for-service basis.
- vi. Undertake scientific research to revive traditional medicines and train the Ayurveds, KabiRaj, and other indigenous healers.

In order to understand and implement these diverse strategies for cost containment, one requires a good understanding of the determinants of health and of the working of the HC market.

2. Problems Related to Access:

Access to HC relates to two categories of problems: "special" or "general". The special problems are faced by particular groups in society- the poor, the slum dwellers, and rural populations including different ethnic groups and the minorities. The general problem is simply to get the kind of care they need when they really need it, irrespective of differences in income, location, or race.

The best way to help the poor is to make cash transfer and let them decide how they want to spend it. Cash transfer to the poor make them better off as compared to in-kind transfer of certain standard of HC (such as providing vouchers to pregnant women to get better prenatal and postnatal care to reduce maternal and infant mortality rates.) General problems also include failure of HC market to match supply and demand of physicians, nurses, hospitals and equipment. Quality of HC facilities at all levels and efficient referral systems.

3. Health levels:

First, there is the concern that health levels differ widely in international comparison of age-sex specific mortality and morbidity rates and life expectancies across countries having almost same level of per capital income. DALYs have been constructed to measure such differences.

Second, that health levels vary greatly among different groups in a country.

Third, over time the introduction of new medical technology has had a significant impact on health, but when we examine differences among populations at a given moment in time, we observe that other socio-economic, cultural and religious variables are much more important than differences in the quantity and quality of HC.

And, finally the human behavior component of many killer diseases like heart attack, cancer and violent deaths from accidents- is very large, and until now HC has not been very successful in altering behavior.

The above discussion of the problems indicates why there is so much concern about HC system and so many proposals for changes in its organization and financing. The promises of the planners and the panaceas of the politicians must be seen against the reality of difficult choices we must make.

The Choices we must make:

1. Health or other Goals?

Is health a basic human right? Even, many people think it is, the realization of that right is always less than complete, because scarce resources have alternative uses.

No country is as healthy as it could be; no nation does as much as for sick as it is technically capable of doing. The grim fact is that no nation is wealthy enough to avoid all avoidable deaths. Many cures are technically possible, not economically feasible.

Is health more important than anything else? Many argue that our unmet health needs with the money "wasted" on cosmetics, cigarettes, pet foods, luxury cars, sophisticated war machine or even multipurpose mobile sets, is possible. But there are other goals

such as beauty, justice, and knowledge, which are also remained unfulfilled because of resource constraints.

For health professionals, the "optimum level" is the highest level technical attainable, regardless of costs, But economists are concerned with "social optimum" such that $SMB \geq SMC$ and SMC is increasing faster than SMB .

Health is one aspect of human living that has to complete resource claims based on it's merits.

2. HC or other health programs?

Curative care vs Preventive measures

- Pollution control
- Safe water and sanitation
- Accident prevention
- Vaccinations
- BCC
- Screening programs
- Anti-drug campaign

Example: Drug interdiction vs drug education

3. Physicians or other health care providers?

Design of health infrastructure where a proportion of mix among various inputs of HC like physicians, nurses, hospital beds, outpatient facilities, lab assistants, diagnostic centers, etc. must be based on the relative prices and their marginal contribution to health. Of course, there is no single right proportion or mix that fits all sizes. It varies across countries and even regions. We have to consider the role of traditional medicine and generic drug production.

4. How much equality? And how to achieve it?

The issue of equity in HC is a more subtle and contentious issue. Based on "need" irrespective of individuals ability to pay.

Horizontal versus vertical equity? McLachlan and Manyard (1986) remarked that----
"equity like beauty is in the mind of the beholder....."

But according to Culyer and Wagstaff (1992), HC ought to be distributed according to need, has so many contested meanings:

- i. need as initial health
- ii. need as a capacity to benefit
- iii. need as expenditure a person ought to have
- iv. need as expenditure required to exhaust capacity to benefit

Equity in finance and delivery of HC is one of the most debated issues among the health professionals and economists today. Equity of access has also many ambiguities in

interpretations. The notion that HC should be available to all, regardless of ability to pay, is growing in the body politics. But we also observe glaring disparities in housing, education, legal services and other important goods and services as well.

5. Today or Tomorrow?

More resources consumed today means less left for investment and increase in future productivity and growth. Investment takes place when a tree is planted, when a student goes to school, when you brush your teeth, as well as when you build a house, a factory or a hospital.

Keeping provision for future is an intertemporal choice and we have already observed that less is kept even for future HC consumption, because present is valued more highly as compared to future.

Volatility in human response to HC is particularly important. Because when health is good, people do not care about health and think that he/she will remain healthy tomorrow as well. But when health is bad, one cares only about health. Hence the opportunity cost of health is state dependent. As a result, most people take irrational decision about health depending on whether he/she is healthy or sick today.

Significant positive correlation between health and education is observed because individuals with relatively low rate of marginal time preference will be more likely self-select to invest more in both health and education.

6. Your life or mine?

Whose life is more valuable? Human capital approach implies that the capacity of medical services to intervene near the point of death is growing rapidly. Such interventions are often extremely costly, but sometimes they work.

Whose life should be saved? The wealthy man's or the poor? Society cannot escape this problem any more than it can avoid facing other choices.

Economics cannot provide final answers to these difficult problems of social priorities, but it can help decision-makers think more rationally or at the margin.

7. The Jungle or Zoo?

“Laissez Faire”

“resolute refusal to accept that society had any responsibility for the causes of human distress”

VS.

“Collective Responsibility”

practiced in extreme form in the “communist” countries where people lose individual freedom for collective benefits; we have seen that those communist countries have dismally failed because they tried to control market through central planning, by trying their “invisible hand” behind their back.

Strict control over a man's behavior might well result in increased life expectancy, but a well-run zoo is still a zoo and not a worthy model for mankind.

Rabbi Hillel more than 1000 years ago said..... "If I am not for myself, who will be for me, but if I am for myself alone, what am I?"

Economics is the service of means, not of ends. It can tell us the consequences of various alternatives, but it cannot make the choice for us. These limitations will be with us always, for economics can never replace morals or ethics.